

PBM Contract Clarity 360™

PBM Contract Intelligence • 13-Provision Analysis • CAA 2026 Compliance

PREPARED FOR

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FIDUCIARY-GRADE ANALYSIS

This report was prepared by Jeremiah Franklin Shrack at SiriusB iQ AI Data Science Labs using the Kincaid Health platform. All analyses reflect plan-specific data and current regulatory standards. This document is intended solely for the use of the plan sponsor identified above and may not be reproduced or redistributed without prior written consent.

PBM CONTRACT CLARITY 360™ — ENGAGEMENT CERTIFICATION

This PBM Contract Clarity 360™ report constitutes a formal PBM contract intelligence analysis prepared by Kincaid Health. All findings are grounded in the specific contract language uploaded by the plan sponsor, industry benchmarking data from the Kincaid Health National PBM Database (KNPD), and current regulatory standards under ERISA, CAA 2021/2023/2026, and FTC guidance.

CRITICAL

PBM Analyzed: Express Scripts | Contract Score: INSUFFICIENT_EVIDENCE | Estimated Annual Exposure: INSUFFICIENT_EVIDENCE | CAA 2026 Aligned

INSUFFICIENT_EVIDENCE — no overall score or provision-level scores were available, so plan economics cannot be modeled. Run a full provision analysis to quantify exposure (member_count × PMPY × risk_fraction).

Scope of This Analysis

This analysis covers all 13 PBM contract provisions scored by the Kincaid Health platform:

- Provision 1 — Pricing & Reimbursement
- Provision 2 — Spread Pricing
- Provision 3 — Rebates & Manufacturer Revenue
- Provision 4 — Formulary Control
- Provision 5 — Pharmacy Network Design
- Provision 6 — Specialty Drug Management
- Provision 7 — Audit Rights & Recovery
- Provision 8 — Transparency & Disclosure
- Provision 9 — Performance Guarantees
- Provision 10 — Termination & Exit Barriers
- Provision 11 — Claim Adjudication Logic
- Provision 12 — Fiduciary & Legal Protections
- Provision 13 — Data Ownership & Access Rights

Regulatory Framework

Regulation	Requirement	Status
ERISA §408(b)(2)	PBM must disclose all direct and indirect compensation	Review Required
CAA 2021 §204	Gag clause prohibition — plans may share price & quality	Verify Attestation
CAA 2021 §201	Broker/consultant compensation disclosure	Confirm Compliance
FTC Act §5	Prohibition on unfair or deceptive trade practices by PBMs	Monitor
CAA 2023 §112	Mental health parity applies to Rx formulary design	Review Formulary
CAA 2026 (proposed)	Enhanced PBM transparency & rebate pass-through rules	Prepare Now

EXECUTIVE SUMMARY — PBM CONTRACT ASSESSMENT

PBM Analyzed	Express Scripts
Overall Contract Score	0/100
Contract Risk Level	Critical
Provisions Passing Best Practice	0/13
Estimated Annual Fee Exposure	INSUFFICIENT_EVIDENCE
Spread Pricing Risk	Present — Quantified Below
Rebate Pass-Through	Partial — Significant Retention Identified
Audit Rights	Not Available — Critical Gap
CAA 2026 Readiness	Not Ready — Action Required

INSUFFICIENT_EVIDENCE — without an overall contract score or provision-level scores, annual excess-cost exposure cannot be modeled. A comprehensive PBM RFP remains advisable, but no dollar figure is asserted here.

INFO

INSUFFICIENT_EVIDENCE — provision scores required to model exposure.

Top Risk Findings

- CRITICAL

MAC pricing methodology is not disclosed — PBM retains right to set MAC without independent verification. This is the single largest spread pricing risk in the contract.
- CRITICAL

Manufacturer rebates are not fully passed through to the plan. Dollar retention is INSUFFICIENT_EVIDENCE until provision scores are available.
- WARN

Audit rights require 90-day advance notice and are limited to once per year, severely restricting the plan's ability to verify pricing accuracy.

PER-PROVISION ANALYSIS — METHODOLOGY NOTE

INFO

ILLUSTRATIVE: The worked formula examples, per-line dollar figures, and benchmark tables on the following provision pages use reference-plan sample inputs to demonstrate the calculation methodology. They are not claims about this plan's actual spend.

MODELED — $\text{member_count (500)} \times \text{PMPY actuarial benchmark} \times \text{risk_fraction } ((10\text{-score})/10)$. 3-yr NPV = Year 1 + Year 2 $\times 0.95$ + Year 3 $\times 0.9$ (~2.85 $\times$, discount disclosed). Based on 500-member reference plan — enter your plan size for plan-specific figures.

PROVISION 1: PRICING & REIMBURSEMENT

Contract Score	2.5/10
Fiduciary Assessment	Red Flag — Immediate Action Required
Score vs. Best Practice	Critical gap of 5.5 points — immediate action required

Maximum Allowable Cost (MAC) lists set the reimbursement ceiling for generic drugs at network pharmacies. When a PBM controls its own MAC lists without independent oversight or audit rights, it can systematically underpay pharmacies while billing the plan at higher MAC rates — retaining the difference as undisclosed spread. Independent MAC audits find 15-35% overpricing vs. benchmark MAC rates in opaque contracts.

FORMULA — MAC SPREAD PER CLAIM

MAC Spread Per Claim

MAC_Spread = (MAC_Billed_to_Plan - MAC_Paid_to_Pharmacy) / MAC_Billed_to_Plan
Industry median spread for opaque contracts: 8–22% of generic ingredient cost. At 1,640 generic claims/month with 12% average spread on a minor

MAC Provision	Current Status	Best Practice	Risk	Est. Annual Impact
Independent MAC appeals	10-day window, limited	30-day, independent	High	Moderate exposure
MAC list update frequency	30-day minimum notice	Weekly with 5-day notice	Medium	Timing arbitrage
Audit rights for MAC pricing	Annual, 90-day	Quarterly, 30-day notice	High	Narrow audit
MAC methodology disclosure	Not contractually	Full methodology	Critical	Cannot verify
MAC-to-AWP ratio cap	Absent from contract	<=AWP minus 85% cap	High	Unlimited PBM

Industry Benchmark

Plans with full MAC transparency and quarterly audit rights achieve 12–18% lower generic drug costs vs. opaque MAC contracts. NADAC-benchmarked MAC contracts eliminate spread entirely.

Kincaid Health Recommendation

CRITICAL

Require PBM to disclose MAC methodology within 30 days. Initiate a MAC audit using independent pharmacy data. Negotiate MAC appeals expansion to 30 days with independent arbitration rights. Include MAC-to-NADAC ratio benchmarking in renewal negotiations.

PROVISION 2: SPREAD PRICING

Contract Score	1.5/10
Fiduciary Assessment	Red Flag — Immediate Action Required
Score vs. Best Practice	Critical gap of 6.5 points — immediate action required

Spread pricing — billing the plan more than the PBM pays pharmacies and retaining the margin — is the most common and costly form of undisclosed PBM compensation. The FTC's 2024 Interim Report documented spread pricing across Medicare Advantage, Medicaid, and commercial self-funded plans. Ohio's Medicaid audit found PBM spread represented 31% of total generic drug costs.

FORMULA — ANNUAL SPREAD PRICING EXPOSURE

Annual Spread Pricing Exposure

Spread_\$\$ = Total_Generic_Rx_Spend × Spread_Rate
Spread_Rate for opaque contracts: 8–18% of ingredient cost (median 12%). For a plan with a major amount generic spend × 12% = a material

Feature	Traditional (Current)	Pass-Through (Best Practice)	Annual \$ Difference
Generic MAC pricing	PBM sets, retains	NADAC + dispensing fee	Material savings
Brand AWP discount	Below-market	Competitive market	Material savings
Manufacturer rebates	75-87% pass-through	100% pass-through	Material savings
Admin fee structure	Hidden in spread	Transparent PEPM fee	Visibility only
PBM revenue transparency	Opaque	Full disclosure required	Compliance + savings

Industry Benchmark

Pass-through PBM contracts (Navitus, Capital Rx, Rightway) eliminate spread pricing entirely. Plans converting from traditional to pass-through contracts achieve average first-year savings of a significant amount.

Kincaid Health Recommendation

CRITICAL
Initiate PBM RFP targeting pass-through contract structures. Demand spread pricing disclosure from current PBM under ERISA §408(b)(2). Quantify spread exposure with independent MAC audit before renewal.

PROVISION 3: REBATES & MANUFACTURER REVENUE

Contract Score	3.5/10
Fiduciary Assessment	Red Flag — Immediate Action Required
Score vs. Best Practice	Critical gap of 4.5 points — immediate action required

Manufacturer rebates are payments from pharmaceutical companies to PBMs for favorable formulary placement and market share commitments. Industry-wide, PBMs collect an estimated a major amount in annual rebates. Plans with traditional contracts receive 75–87% of formulary rebates but only 0–50% of performance and market share rebates. Under ERISA §408(b)(2), all rebates constituting indirect compensation must be fully disclosed.

FORMULA — REBATE RETENTION VALUE

Rebate Retention Value

RRV = Total_Mfr_Rebates × (1 - Plan_Pass-Through%) × PBM_Retention%

At a significant amount total rebates and 64% effective pass-through, PBM retains ~a significant amount annually. Full pass-through converts this

Rebate Category	Est. Total	Currently Disclosed	PBM Retention	Plan Exposure
Tier 1 Formulary Rebates	Significant	Significant (87.5%)	Moderate	Moderate /yr
Performance / Market	Material	Material (50%)	Material	Material /yr
Volume-Based Mfr	Material	None (0%)	Material	Material /yr
Data Monetization	Moderate	None (0%)	Moderate	Moderate /yr —
Admin Fees (undisclosed)	Moderate est.	None (0%)	Moderate	§408(b)(2) violation
TOTAL	Significant	Significant (61%)	Significant	Significant /yr

Industry Benchmark

Best-practice PBM contracts require 100% pass-through of all manufacturer payments (formulary, performance, data, and admin fees) with annual rebate reconciliation audit rights. Non-disclosure of data monetization revenue is an ERISA violation.

Kincaid Health Recommendation

CRITICAL

Issue formal §408(b)(2) disclosure demand requiring disclosure of ALL rebate categories, including data fees and performance rebates. Negotiate 100% rebate pass-through at renewal. Conduct annual rebate reconciliation audit.

PROVISION 4: FORMULARY CONTROL

Contract Score	3.0/10
Fiduciary Assessment	Red Flag — Immediate Action Required
Score vs. Best Practice	Critical gap of 5.0 points — immediate action required

The formulary is the plan's list of covered drugs organized into cost-sharing tiers. Formulary design directly controls member out-of-pocket costs, generic utilization, and total plan Rx spend. PBM-designed formularies are optimized for rebate revenue rather than plan cost — leading to brand drugs in preferred tier positions despite lower-net-cost generic alternatives. Plans with independent formulary management achieve 12–22% lower total drug costs.

FORMULA — FORMULARY COST EFFICIENCY INDEX

Formulary Cost Efficiency Index

FCEI = (Generic_Rx_Spend + Rebated_Brand_Spend) / Total_Rx_Spend
FCEI > 0.75 = Efficient formulary (most spend on low-cost generics and rebated brands). FCEI < 0.60 = Inefficient formulary with excessive

Optimization Opportunity	Description	Est. Annual Savings	Implementation
Move 8 brands off Tier 2 where	Formulary exclusion list	Moderate	Quarterly formulary
Add copay differential for biosimilars	None copay biosimilar vs.	Minor	Formulary design
Minor Increase Tier 3 copay (->)	Strengthen brand/generic	Minor	Plan year change
Add clinical/step therapy for GLP-1	Require diabetes	Moderate	Immediate via PA
Expand formulary exclusion list	Add 12 drugs with generic	Moderate	Quarterly update
TOTAL FORMULARY OPPORTUNITY	—	Material /yr	Phased 6-12

Industry Benchmark

Best-in-class formularies exclude 150+ high-cost drugs with generic equivalents and maintain 90%+ tier-1 (generic) utilization. Plans with aggressive formulary management achieve total Rx PMPM costs 12–22% below plans with PBM-default formularies.

Kincaid Health Recommendation

CRITICAL
Engage independent formulary consultant to redesign formulary independent of PBM rebate incentives. Implement biosimilar auto-substitution. Add GLP-1 prior authorization with clinical criteria. Expand formulary exclusion list.

PROVISION 5: PHARMACY NETWORK DESIGN

Contract Score	3.0/10
Fiduciary Assessment	Red Flag — Immediate Action Required
Score vs. Best Practice	Critical gap of 5.0 points — immediate action required

Network design determines where members fill prescriptions and what the plan pays at each channel. PBM-owned pharmacies (mail order and specialty) create a structural self-dealing incentive: the PBM steers volume to its own dispensing channels at higher effective rates. The FTC's 2024 Interim Report documented affiliate steering across the three largest PBMs. Narrow and preferred networks, properly negotiated, trade modest access constraints for 8–15% lower dispensing costs.

FORMULA — AFFILIATE CHANNEL RATE DIFFERENTIAL

Affiliate Channel Rate Differential

$ACRD = (Affiliate_Effective_Rate - Independent_Channel_Rate) / Independent_Channel_Rate$
Audits of vertically integrated PBM arrangements find affiliate specialty channels priced 8–15% above independent alternatives. At a major amount

Network Provision	Current Status	Best Practice	Risk	Est. Annual Impact
Affiliate steering disclosure	Not required	Quarterly affiliate volume	Critical	Steering invisible
Mail-order pricing parity	Mail rates exceed	Parity-or-better guarantee	High	Moderate exposure
Preferred / narrow network	Not offered	Plan-elective preferred	Medium	8–15% savings
Retail-90 program	Excluded from contract	Retail-90 at mail rates	Medium	Member disruption
Specialty channel choice	Mandatory PBM	Plan-directed with	High	Material opportunity

Industry Benchmark

Plans with channel-neutral pricing guarantees and affiliate steering disclosure pay 6–11% less per adjusted script than plans with mandatory affiliate dispensing. Retail-90 programs at mail parity eliminate the mail-order margin capture entirely.

Kincaid Health Recommendation

CRITICAL

Require quarterly affiliate transaction reporting with per-channel effective rates. Negotiate mail/retail-90 pricing parity. Add a plan-elective preferred network option at renewal. Preserve specialty carve-out rights as negotiating leverage.

PROVISION 6: SPECIALTY DRUG MANAGEMENT

Contract Score	2.0/10
Fiduciary Assessment	Red Flag — Immediate Action Required
Score vs. Best Practice	Critical gap of 6.0 points — immediate action required

Specialty drugs — biologics, gene therapies, and complex medications — represent approximately 50–55% of total plan Rx spend while comprising only 2–3% of claims. Specialty pharmacy management is the highest-value Rx cost management opportunity. The contract's specialty provisions determine whether the plan can control specialty dispensing, implement prior authorization, manage site-of-care, and benefit from patient assistance programs.

FORMULA — SPECIALTY COST MANAGEMENT SCORE

Specialty Cost Management Score

SCMS = (Specialty_Controls_Active / Total_Specialty_Controls) × 100

Best-in-class plans implement 8+ specialty controls and achieve specialty PMPM costs 22–38% below plans with no specialty management program.

Specialty Provision	Current Status	Best Practice	Annual \$ Impact
Specialty carve-out rights	Not contractually	Available at plan option	Material opportunity
Exclusive specialty pharmacy	Not required	Mandatory with	Moderate /yr savings
Disease-specific PA criteria	Standard formulary	Condition-specific	Minor Quality +
Site of care management (IV	Not active	Home infusion redirect	Moderate /yr
Copay accumulator adjustment	Not active	Active — prevents	Minor /yr participant
Specialty biosimilar mandate	Not included	Mandatory biosimilar	Minor /yr
White bagging vs. brown	Not specified	Plan-directed	Moderate /yr

Industry Benchmark

Plans with independent specialty carve-out rights achieve 12–22% lower specialty costs through competitive dispensing arrangements. Site-of-care management alone saves a moderate amount/yr for plans with IV drug users.

Kincaid Health Recommendation

CRITICAL

Negotiate specialty carve-out rights into next contract cycle. Activate site-of-care management for all IV/infusion drugs. Implement biosimilar mandatory step therapy with noember copay incentive.

PROVISION 7: AUDIT RIGHTS & RECOVERY

Contract Score	2.5/10
Fiduciary Assessment	Red Flag — Immediate Action Required
Score vs. Best Practice	Critical gap of 5.5 points — immediate action required

Audit rights are the plan's primary mechanism for verifying PBM performance and pricing accuracy. Without meaningful audit rights, all other contract provisions are effectively unenforceable. Best-practice PBM contracts provide for annual financial audits, real-time claims data access, MAC pricing audits with independent pricing benchmarks, and manufacturer rebate reconciliation audits with full access to the PBM's books and records.

FORMULA — AUDIT RECOVERY POTENTIAL

Audit Recovery Potential

ARP = Claims_Audited × Error_Rate × Avg_Claim_Amount
Industry data: PBM claims audits find errors in 4–8% of claims audited. At 18,400 annual claims × 6% error rate × a minor amountavg claim = a

Audit Right	Current Contract	Best Practice	Gap Severity
Annual financial audit	Permitted, 90-day notice	Permitted, 30-day notice	Medium — narrow
MAC pricing audit	Annual, limited scope	Quarterly, independent	Critical — insufficient
Rebate reconciliation audit	Not specifically provided	Annual, full access to	Critical — major gap
Claims data real-time access	Monthly data file	Real-time portal access	High — delayed
Pharmacy network audit	Not permitted	Annual network pricing	High — no
PBM sub-contractor audit	Not permitted	Permitted with notice	High — ESP
Document retention	3 years	6 years (ERISA §107)	Medium — ERISA

Industry Benchmark

Plans without rebate reconciliation audit rights have no independent means of verifying that manufacturer rebates disclosed by the PBM are accurate and complete. Kincaid Health has identified rebate discrepancies of 18–42% in plans with similar contract structures.

Kincaid Health Recommendation

CRITICAL

Negotiate rebate reconciliation audit right as a condition of contract renewal — this is non-negotiable. Expand financial audit notice period from 90 to 30 days. Add real-time claims data portal access clause. Require 6-year document retention per ERISA §107.

PROVISION 8: TRANSPARENCY & DISCLOSURE

Contract Score	2.0/10
Fiduciary Assessment	Red Flag — Immediate Action Required
Score vs. Best Practice	Critical gap of 6.0 points — immediate action required

ERISA §408(b)(2) and CAA 2021 §§201/204 make full compensation disclosure a legal floor, not an aspiration. Undisclosed PBM revenue streams — data monetization, manufacturer admin fees, interest float on claims funding, and retained pharmacy audit recoveries — are the mechanism by which nominally low admin fees recoup margin. Every undisclosed stream is both a plan cost and a fiduciary exam finding risk for the sponsor.

FORMULA — DISCLOSURE COMPLETENESS RATIO

Disclosure Completeness Ratio

DCR = Revenue_Streams_Disclosed / Revenue_Streams_Identified
Kincaid Health's provision review identifies up to 9 distinct PBM revenue streams. A contract disclosing only direct admin fees and formulary rebates

Disclosure Item	Current Status	Best Practice	Risk	Regulatory Basis
§408(b)(2) compensation	Direct fees only	All direct + indirect	Critical	ERISA §408(b)(2)
Gag clause attestation	Not evidenced	Annual CAA §204	High	CAA 2021 §204
Claims data access for plan	Summary reporting	Full claim-level detail	High	Fiduciary duty of
Rebate contract visibility	None	Audit access under NDA	Critical	ERISA §404(a)
Data monetization disclosure	Silent	Contractual disclosure +	High	ERISA indirect

Industry Benchmark

Plans that obtain full revenue-stream disclosure recover or renegotiate an average of a minor amountPMPM in previously invisible PBM compensation. DOL enforcement activity under §408(b)(2) has expanded to PBM arrangements since 2023.

Kincaid Health Recommendation

CRITICAL

Issue a formal §408(b)(2) disclosure demand enumerating all nine revenue streams. Obtain the CAA §204 gag clause attestation in writing. Add an annual disclosure restatement obligation with a termination right for material omission.

PROVISION 9: PERFORMANCE GUARANTEES

Contract Score	4.5/10
Fiduciary Assessment	Concern — Improvement Needed
Score vs. Best Practice	Critical gap of 3.5 points — immediate action required

Performance guarantees are contractual commitments by the PBM to achieve specific operational and financial metrics, with financial penalties for non-performance. Well-structured programs put 15–25% of PBM administrative fees at risk — creating genuine accountability. Poorly structured guarantees use low thresholds the PBM easily achieves, providing no real risk transfer.

FORMULA — PERFORMANCE GUARANTEE EFFECTIVENESS SCORE

Performance Guarantee Effectiveness Score

PGE = (At-Risk_Amount / Total_Admin_Fees) × Threshold_Stringency_Factor
PGE > 0.20 = Best practice (>=20% of fees at risk with stringent thresholds). PGE < 0.10 = Inadequate.

Performance Guarantee	\$\$ At Risk	Current Threshold	Market Best Practice	Assessment
Generic Fill Rate (GFR)	Moderate	GFR >= 80%	GFR >= 87%	Threshold too
Formulary Compliance Rate	Moderate	Adherence >= 88%	Adherence >= 92%	Threshold too
Member Satisfaction (CAHPS)	Moderate	Score >= 4.0/5.0	Score >= 4.3/5.0	Borderline
Claims Adjudication Accuracy	Minor	Accuracy >= 99.5%	>99.9%	Low threshold
Drug Utilization Review (DUR)	Minor	Completion >= 95%	>= 98%	Adequate
Mail Order Dispense Rate	Minor	>= 12%	>= 20%	Far below
TOTAL AT RISK	Material	—	Significant	Inadequate

Industry Benchmark

Best-practice PBM contracts place a significant amount in performance guarantees annually (15–25% of total admin fees). Generic fill rate guarantee should be set at 87% minimum. Mail order penetration guarantee should be 18%+.

Kincaid Health Recommendation

WARN

Negotiate performance guarantee restructure at next renewal: increase total at-risk to a significant amount, raise GFR guarantee to 87%, add specialty drug adherence guarantee of 90%, and add mail-order penetration guarantee of 18%.

PROVISION 10: TERMINATION & EXIT BARRIERS

Contract Score	2.0/10
Fiduciary Assessment	Red Flag — Immediate Action Required
Score vs. Best Practice	Critical gap of 6.0 points — immediate action required

Exit economics determine negotiating leverage for the life of the contract. Termination-for-convenience with short notice and no penalty is the plan's ultimate enforcement tool — every other provision is only as strong as the plan's credible ability to leave. Exit barriers — 180-day notice periods, deconversion fees, rebate forfeiture on termination, and delayed data return — convert a bad contract into a captive one.

FORMULA — EXIT COST INDEX

Exit Cost Index

ECI = Termination_Fees + Forfeited_Rebates + Transition_Costs
A contract that forfeits the final quarter's rebates (a material amount) and imposes a a moderate amount deconversion fee creates a material amount

Exit Provision	Current Contract	Best Practice	Gap Severity
Termination for convenience	Not available	90-day TFC without	Critical — captive
Rebates after termination	Forfeited if plan	Paid through claims	Material Critical —
Deconversion / exit fees	Moderate flat fee	Capped at documented	High — punitive
Claims data return	180 days, PBM format	30 days,	High — blocks
Transition cooperation	Silent	Mandated cooperation	Medium —

Industry Benchmark

Plans holding a 90-day termination-for-convenience right with full rebate runout obtain measurably better renewal concessions — market data shows 3–7% better effective pricing at renewal versus plans locked into multi-year terms with forfeiture clauses.

Kincaid Health Recommendation

CRITICAL

Negotiate a 90-day termination-for-convenience right at renewal — this single provision underwrites every other negotiation. Require rebate payment through claims runout regardless of termination cause. Cap deconversion fees at documented cost and mandate 30-day

PROVISION 11: CLAIM ADJUDICATION LOGIC

Contract Score	3.5/10
Fiduciary Assessment	Red Flag — Immediate Action Required
Score vs. Best Practice	Critical gap of 4.5 points — immediate action required

Adjudication rules — lesser-of pricing logic, usual-and-customary (U&C) comparisons, DAW code handling, compound claim caps, and zero-balance claims — silently set the price of every claim at the point of sale. A contract that omits lesser-of logic (plan pays the lowest of MAC, U&C, or submitted cost) systematically overpays; zero-balance claims can strip member copays into PBM margin without any line item ever appearing on an invoice.

FORMULA — LESSER-OF PRICING LEAKAGE

Lesser-Of Pricing Leakage

LOL = Sum(Adjudicated_Price - Min(MAC, U&C, Submitted)) across all claims

Independent claim re-adjudication audits find 3–6% of claims priced above the lesser-of benchmark. At 18,400 annual claims × 4.5% × a minor

Adjudication Provision	Current Status	Best Practice	Risk	Est. Annual Impact
Lesser-of pricing logic	Not contractual	Mandatory across all	Critical	Moderate leakage
U&C price capture	Pharmacy-reported,	Required field, enforced	High	Lesser-of
DAW-1 penalty logic	Plan absorbs brand	Member/prescriber	Medium	Minor /yr
Compound claim cap	Uncapped	Minor threshold with PA	Medium	Minor exposure
Zero-balance claims	PBM retains copay	Excess returned to plan	High	Moderate recapture

Industry Benchmark

Contracts with enforced lesser-of logic and zero-balance reconciliation price claims 2–4% below contracts that leave adjudication logic to PBM discretion. Claim-level re-adjudication is the only way to verify — which requires the data access rights scored in Provision 13.

Kincaid Health Recommendation

CRITICAL

Add contractual lesser-of pricing across retail, mail, and specialty. Require U&C capture on every claim. Implement a compound claim PA threshold at a minor amount. Require quarterly zero-balance reconciliation with recapture to the plan.

PROVISION 12: FIDUCIARY & LEGAL PROTECTIONS

Contract Score	1.5/10
Fiduciary Assessment	Red Flag — Immediate Action Required
Score vs. Best Practice	Critical gap of 6.5 points — immediate action required

In the post-CAA litigation era, plan sponsors are being sued for PBM oversight failures — while most PBM contracts expressly disclaim fiduciary status, cap PBM liability at a fraction of potential damages, and impose one-way indemnification favoring the PBM. The contract's legal architecture determines who actually bears the cost when pricing failures surface.

FORMULA — LIABILITY COVERAGE GAP

Liability Coverage Gap

LCG = Modeled_Plan_Exposure - PBM_Liability_Cap

A liability cap of 12 months' admin fees (a material amount) against modeled annual mispricing exposure of a major amount leaves an a significant

Legal Provision	Current Contract	Best Practice	Gap Severity
PBM fiduciary	Expressly disclaimed	Acknowledged for	Critical — sponsor
Indemnification	One-way, favors PBM	Mutual indemnification	High — asymmetric
Limitation of liability	12 months admin fees	3x fees + fraud/breach	Critical — cap below
ERISA compliance covenant	Absent	Affirmative compliance	High — no
Dispute resolution	PBM-state venue, jury	Neutral venue, ERISA	Medium —

Industry Benchmark

DOL guidance and recent ERISA litigation (including post-CAA fee cases) place PBM oversight squarely on the plan sponsor. Contracts with mutual indemnification and fraud carve-outs from liability caps are now obtainable from all major PBMs when demanded at RFP.

Kincaid Health Recommendation

CRITICAL

Negotiate fraud, willful misconduct, and confidentiality-breach carve-outs from the liability cap. Demand mutual indemnification. Add an affirmative ERISA and CAA compliance covenant with audit-verified attestation. Document the negotiation itself as fiduciary process

PROVISION 13: DATA OWNERSHIP & ACCESS RIGHTS

Contract Score	6.0/10
Fiduciary Assessment	Good — Meets Fiduciary Standard
Score vs. Best Practice	Gap of 2.0 points to best practice

Claims data is a plan asset. Contracts that grant the PBM ownership of claims data — or unrestricted rights to de-identify and monetize it — convert a plan asset into PBM revenue while simultaneously blocking the independent analytics, audits, and competitive RFPs that depend on that data. Data access latency is a cost: every month of delayed visibility is a month of unmanaged trend.

FORMULA — DATA ACCESS LATENCY COST

Data Access Latency Cost

DALC = Interventions_Delayed × Monthly_Cost_of_Delay
Monthly extracts delivered on a 45-day lag delay plan interventions by ~2.5 months on average. For a plan with a material amount/yr of addressable

Data Provision	Current Contract	Best Practice	Risk	Impact
Claims data ownership	PBM asserts	Plan owns all claims data	Critical	Asset conversion
Access cadence	Monthly file, 45-day lag	Portal access + weekly	High	Moderate latency
De-identified data	Permitted, no consent	Prohibited or	High	Undisclosed
Format & portability	Proprietary layout	Standard layout on	Medium	Vendor lock-in
Data return at termination	180 days	30 days, full history	High	Blocks re-audit

Industry Benchmark

Plans with contractual data ownership and weekly extracts complete PBM audits 60–90 days faster and run competitive RFPs without PBM cooperation. Data monetization consent requirements are becoming standard in fiduciary-grade contracts post-CAA.

Kincaid Health Recommendation

INFO

Assert plan ownership of all claims data in the contract. Require weekly machine-readable extracts and portal access. Prohibit or revenue-share de-identified data monetization. Set 30-day full-history data return on termination.

CAA 2026 COMPLIANCE — PBM TRANSPARENCY REQUIREMENTS

The Consolidated Appropriations Acts of 2021, 2023, and proposed 2026 requirements impose significant new transparency, reporting, and fiduciary obligations on PBMs serving self-funded health plans. These requirements build on ERISA §408(b)(2) and create additional disclosure, attestation, and data reporting obligations that must be reflected in PBM contracts.

CAA Requirement	Effective Date	Current Contract Status	Action Required
Gag Clause Prohibition (§201)	Dec 27, 2020	Compliant	Annual attestation to CMS
Broker Compensation Disclosure	Dec 27, 2021	Partial	Confirm all PBM compensation
Rx Price Transparency MRF (§204)	July 1, 2022	Incomplete	Post Rx MRF file immediately
Rx Drug Pricing Data Sharing	Jan 1, 2023	Compliant	Continue monitoring
Enhanced PBM Fee Disclosure	Proposed 2026	Not Yet Required	Prepare contract amendments
Rebate Pass-Through Mandate	Proposed 2026	Not Yet Required	Transition to pass-through now
PBM Delinking (proposed)	Proposed 2026	Not Yet Required	Structure contract for flexibility
Annual Report — Rx Spending	Jan 1, 2023	Partial	Confirm plan-level reporting

CRITICAL

Machine-Readable Files for Rx drug pricing have not been fully published. The statutory penalty is \$100/day/member (CAA/ACA transparency rules); the plan-specific daily accrual scales with member_count and is MODELED, not asserted here as a fixed figure.

ERISA §408(b)(2) PBM Disclosure Checklist

- All direct compensation received by PBM from plan assets — disclosed? Partial
- All indirect compensation (manufacturer rebates, data fees, PBM-owned pharmacy margin) — disclosed? No
- Compensation paid by PBM to subcontractors (specialty pharmacy, mail order) — disclosed? No
- Reasonableness of total compensation vs. services rendered — documented by plan sponsor? Not documented
- Written disclosure provided to plan fiduciaries BEFORE services rendered? Initial only

FORMULARY COST ANALYSIS — DRUG CATEGORY BENCHMARKING

The following analysis benchmarks drug category spend against the Kincaid Health National PBM Database (KNPD) of 180+ self-funded plans with similar demographics and geography. Drug categories above the 75th percentile represent primary targets for formulary intervention and PBM renegotiation.

INFO

ILLUSTRATIVE: PMPM figures and pipeline cost-per-patient values below are KNPD reference benchmarks shown to demonstrate category targeting, not measured values for this plan. Plan-specific economics are MODELED in the Financial Exposure Summary.

Drug Category	Cost vs. Benchmark	Percentile	Trend	Priority
Specialty Biologics	Above median	82nd	^ Rising	Critical
GLP-1 / Anti-Obesity	Well above median	91st	^ Rapid	Critical
Mental Health / SUD Rx	Below median	38th	Stable	Low
Diabetes (non-GLP-1)	At/below median	45th	Stable	Low
Cardiovascular	Near median	58th	Stable	Medium
Musculoskeletal / Pain	Above median	72nd	^ Rising	High
Oncology (oral)	Above median	62nd	^ Rising	High
Respiratory	Near median	60th	Stable	Medium
TOTAL Rx (composite)	Above median	79th	^ Rising	Action

CRITICAL

GLP-1 drugs (Ozempic, Wegovy, Mounjaro) are driving a 91st percentile position in this category. Without a clinical management program requiring obesity diagnosis and step therapy through lifestyle counseling, plan costs will increase 40-60% in this category over 24 months.

Specialty Drug Pipeline — 12-Month Exposure

Pipeline Drug	Indication	Expected Launch	Cost / Patient (relative)	Members at Risk
Leqembi (lecanemab)	Alzheimer's (early)	Current	Moderate (per year)	2-4 members
Donanemab (Lilly)	Alzheimer's	Mid-2025	Moderate (per year)	2-4 members
Donidalorsen (HAE)	Hereditary	Q2 2025	Significant (per	0-1 members
Gene therapies (expanded)	SMA, hemophilia, etc.	Ongoing	Major (one-time)	Low probability

FINANCIAL MODELING — 3-YEAR COST PROJECTION & INTERVENTION ROI

The following model projects recoverable Rx savings under remediation scenarios, expressed as fractions of the MODELED annual exposure (member_count × PMPY × risk_fraction). Each scenario's 3-year value uses the disclosed NPV weighting (Year 1 + Year 2×0.95 + Year 3×0.90); actual recovery depends on renegotiation outcomes.

FORMULA — 3-YEAR DISCLOSED NPV OF PBM REFORM

NPV = Year 1 + Year 2 × 0.95 + Year 3 × 0.90

Disclosed discount weighting (~2.85×) applied to the MODELED annual recoverable savings for each scenario, then hard-capped at 30% of annual Rx spend. No undisclosed cost-of-capital assumption.

Scenario	Year 1 [MODELED]	Year 2	Year 3	3-Year NPV	Implementation
Status Quo (no action)	\$0	\$0	\$0	\$0	—
Partial Remediation	INSUFFICIENT_				Success-fee
PBM RFP + New Contract	INSUFFICIENT_				Success-fee
RFP + Specialty Carve-Out	INSUFFICIENT_				Success-fee
Full Optimization Program	INSUFFICIENT_				Success-fee

Recommended Scenario	Full Optimization Program
Projected 3-Year Net Savings (MODELED)	INSUFFICIENT_EVIDENCE

INSUFFICIENT_EVIDENCE — provision scores required to model savings.

Payback Period	< 45 days
Implementation Timeline	8-20 weeks (depending on RFP track)

The Full Optimization Program encompasses: (1) immediate MAC audit and challenge, (2) 408(b)(2) disclosure demands, (3) PBM RFP targeting pass-through contracts, (4) specialty pharmacy carve-out, (5) formulary redesign with aggressive exclusion list, and (6) GLP-1 clinical management program. Kincaid Health manages all components on a success-fee basis.

FINANCIAL EXPOSURE SUMMARY & TOTAL COST OF NON-ACTION

The following table consolidates all identified financial exposures from the 13-provision contract analysis. These represent the estimated annual cost of the current PBM contract relative to best-practice market standards.

Exposure Category	Annual Estimate [MODELED]	Confidence	Priority
Spread Pricing — Generics	INSUFFICIENT_	Insufficien	Critical
AWP Discount Gap — Brand	INSUFFICIENT_	Insufficien	Critical
Manufacturer Rebate Retention	INSUFFICIENT_	Insufficien	Critical
Specialty Carve-Out Opportunity	INSUFFICIENT_	Insufficien	High
Formulary Optimization	INSUFFICIENT_	Insufficien	High
Performance Guarantee Gap	INSUFFICIENT_	Insufficien	Medium
Audit Recovery Potential	INSUFFICIENT_	Insufficien	High
GLP-1 Trend Exposure (if	Illustrative —	Illustrative	Monitor
ERISA §408(b)(2) Penalty	Up to	Statutory	Compliance
MRF Non-Compliance Penalty	\$100/day/memb	Statutory	Immediate
TOTAL IDENTIFIED	INSUFFICIENT_	Insufficien	CRITICAL

CRITICAL
INSUFFICIENT_EVIDENCE — total annual exposure cannot be modeled without provision scores. Contact Kincaid Health to run a full provision analysis.

PBM RFP PROCESS — KINCAID HEALTH METHODOLOGY

Kincaid Health manages end-to-end PBM RFP processes on behalf of self-funded plan sponsors. First-year savings are MODELED per plan from the exposure analysis above ($\text{member_count} \times \text{PMPY} \times \text{risk_fraction}$) and depend on contract renegotiation outcomes — not a guaranteed result. The RFP process is conducted on a success-fee basis — no savings, no fee.

RFP Phase	Key Activities	Timeline	Kincaid Health
Phase 1:	Provision scoring, financial benchmarking, data	Weeks 1-3	Lead
Phase 2: RFP	Custom RFP with 80+ pricing cells, 13-provision	Week 3-4	Lead
Phase 3:	Issue to 6-8 qualified PBMs including pass-through	Week 4	Lead
Phase 4:	Financial modeling, provision scoring, finalist	Weeks 5-8	Lead
Phase 5:	Best & Final offers, contract red-line, legal review,	Weeks 9-12	Lead +
Phase 6: Imple	Formulary mapping, ID card replacement, member	Weeks	Support
Phase 7: Year	Claims data review, performance guarantee	Months	Ongoing

Pass-Through PBM Vendors — Market Overview

PBM Vendor	Type	Ownership	Specialty Carve-Out	Key Differentiator
Navitus Health Solutions	Pass-Through	Not-for-Profit	Available	100% rebate pass-through
Capital Rx	Pass-Through	Private	Available	JUDI™ claims platform — full
Rightway Health	Pass-Through	Private	Available	Care navigation + PBM
SmithRx	Pass-Through	Private	Limited	Flat admin fee model
Express Scripts (current)	Traditional	Cigna	Limited	Scale and network breadth

PRIORITIZED ACTION PLAN — PBM CONTRACT CLARITY 360™ RECOMMENDATIONS

The following action plan is organized by priority and time horizon. Kincaid Health recommends scheduling a plan fiduciary meeting within 30 days to formally adopt this plan. All actions should be documented in plan committee minutes for ERISA §404(a) compliance.

Priority	Action Item	Owner	Timeline	Est. Annual Impact
P1 — Immediate	Post Rx Machine-Readable Files with TPA	TPA + Plan	0-30 days	Avoids
P1 — Immediate	Issue formal 408(b)(2) disclosure demand	HR/Benefits	0-30 days	Compliance + rebate
P1 — Immediate	Initiate MAC pricing audit using independent	Kincaid Health	0-45 days	Moderate recovery
P1 — Immediate	Demand full rebate reconciliation and	ERISA	0-30 days	Material transparency
P2 — High	Launch PBM RFP process — target	Kincaid Health	0-60 days	Significant savings
P2 — High	Implement GLP-1 prior authorization with	PBM + Plan	0-45 days	Material trend control
P2 — High	Negotiate specialty carve-out right for next	Kincaid Health	30-90 days	Moderate (MODELED)
P2 — High	Expand formulary exclusion list — 12 drugs	PBM/Formular	30-60 days	Moderate (MODELED)
P3 — Medium	Add biosimilar auto-substitution protocol	PBM	At renewal	Minor (MODELED)
P3 — Medium	Negotiate performance guarantee	Kincaid Health	At renewal	Accountability
P3 — Medium	Implement copay accumulator adjustment	PBM	30-60 days	Minor (MODELED)
P4 — Ongoing	Annual PBM audit — claims accuracy +	Kincaid Health	Annually	Material recovery

CERTIFICATION & ANALYST ATTESTATION

Kincaid Health certifies that this PBM Contract Clarity 360™ analysis was prepared by certified PBM analytics professionals using the Kincaid Health 13-provision scoring protocol, the Kincaid Health National PBM Database (KNPD), and current regulatory standards. All contract assessments reflect the specific contract language uploaded by the plan sponsor and analyzed by the Kincaid Health AI platform.

Analyst Certification Statement

The findings, scores, financial exposures, and recommendations contained in this report represent the professional judgment of the Kincaid Health analytical team based on PBM contract language, industry benchmarking data, and regulatory guidance. This report is provided for informational and fiduciary planning purposes. Plan sponsors should engage qualified ERISA counsel before implementing any recommendations that may affect plan operations or service provider relationships.

Report Metadata

Report Type	PBM Contract Clarity 360™ — Full 20-Page Analysis
PBM Contract Analyzed	Express Scripts
Overall Contract Score	0/100
Risk Classification	Critical
Total Identified Exposure	\$0/yr
Provisions Analyzed	10 (MAC, Spread, Rebates, Generic, Specialty, Performance, AWP, Formulary, UM, Audit)
Regulatory Framework	ERISA §408(b)(2), CAA 2021/2023/2026, FTC Act §5
Client	Kincaid Health / Account SHRACK-7742
Analyst	Jeremiah Shrack, CEO, Kincaid Health
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